

Patient name: **Mr Joao Victor Creste Dias De Souza**
Patient ID:
Born: **Oct 17 1992** Gender: **Male**
Exam date: **Mar 26 2026 18:12** Accession no: **NV1X01005196090**

Radiology Report

Report Date: Apr 02 2026 20:25:00

Patient ID:

Mr Joao Victor Creste Dias De Souza

Date of Birth: Oct 17 1992

Gender: Male

Address:

Accession no: NV1X01005196090

Exam date: Mar 26 2026 18:12

Exam: MRI Spine cervical

Referring Physician: 1 MUHAMMAD NAJIM

Requesting Department:

Observation:

Clinical Information:

History of cervical disc prolapse in 2022. Right arm numbness and moderate neck pain. No arm weakness. Query right cervical radiculopathy.

Technique:

MRI cervical spine without intravenous contrast.

Comparisons:

No previous imaging available for comparison at the time of reporting.

Findings:

Loss of the normal cervical lordosis. Mid-cervical spondylotic change with shallow disc osteophyte complexes from C3/4 to C6/7. Mild spinal canal narrowing at these levels with mild indentation of the cervical cord. Cervical cord signal remains normal, with no cord atrophy or myelopathy.

Uncovertebral hypertrophy is most pronounced at C5/6 and C6/7, right more than left. Mild bilateral neural foraminal narrowing at these levels, more pronounced on the right, with encroachment on the exiting right C6 and right C7 nerve roots. Mild left foraminal narrowing is also present at these levels. No severe spinal canal stenosis.

Vertebral body heights are maintained. No focal marrow oedema or aggressive osseous lesion. Craniocervical junction and visualised paraspinal soft tissues are unremarkable. Otherwise, unremarkable appearances.

Conclusion/Recommendations:

Mid-cervical degenerative change with shallow disc osteophyte complexes from C3/4 to C6/7 causing mild canal narrowing and mild cord indentation, without cord signal abnormality. Right more than left

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uncovertebral hypertrophy at C5/6 and C6/7 with bilateral foraminal narrowing, greater on the right, and encroachment on the exiting right C6 and right C7 nerve roots. Please correlate clinically for right-sided radiculopathy. Spinal review can be considered if the clinical findings are relevant to these appearances.

Information for Patients:

A copy of your report has been sent to your referring clinician or your NHS GP if you are a self-referring patient. We advise that you speak with them regarding the clinical detail within the report

Information for Referring Clinicians only:

Should you require any clinical clarification on the report please contact InHealth Reporting Limited:
Tel: 0370 7749520 Email: INL.Inhealthreporting@NHS.net

Dr Mohsen Alkmeshti
Consultant Radiologist
Registration Number: GMC 7096445
Apollo Radiology International UK

Transcriptionist:

Primary Radiologist: C7096445 Mohsen Alkmeshti
Secondary Radiologist:

Report Status: Final Result